PCOD Xpert

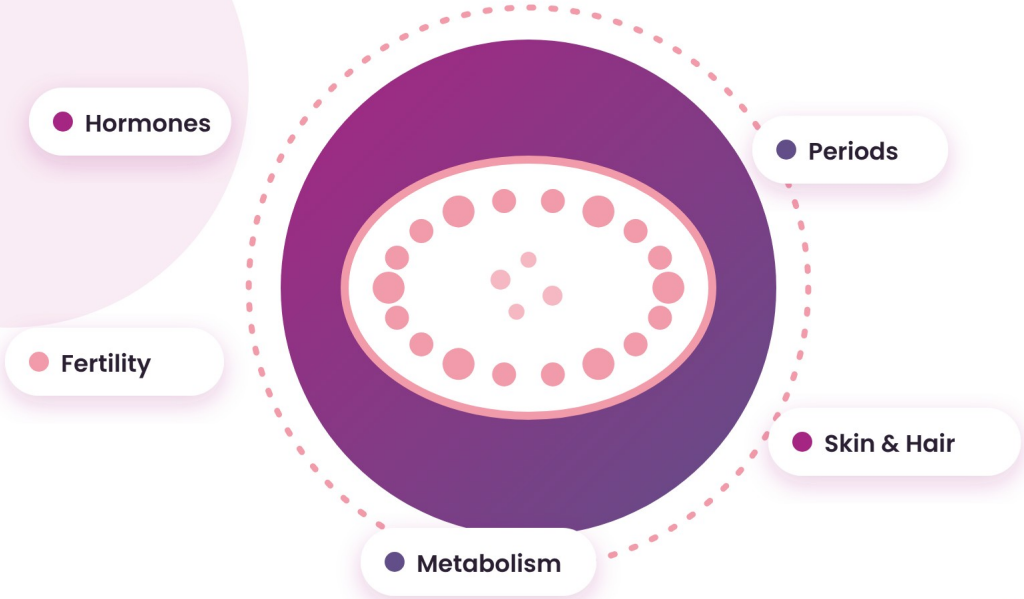
Hope Blooms from Within

PATIENT GUIDE

Everything About

PCOD & PCOS

A simple, illustrated guide for women, teens and families



Hope Blooms from Within

START HERE

Welcome

If you have been told you have **PCOD or PCOS**, you are not alone. About **1 in 5** women of reproductive age lives with it. It is common, it is manageable, and understanding it is the first step.

How to use this booklet

- Read one page at a time. Each page explains one idea with a picture.
- Use the **symptom decoder** (page 13) to understand what your body is telling you.
- Fill in the **tracker** (page 22) and bring it to your appointment.

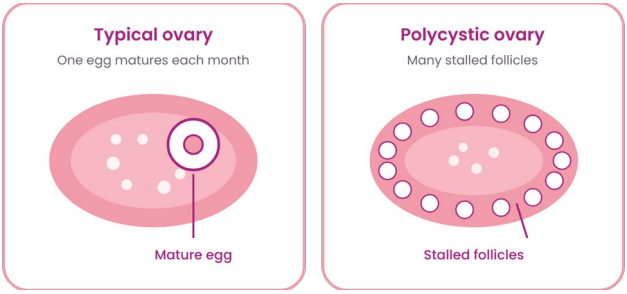
> **For teens and parents**
> Irregular periods are common in the first 2 to 3 years after the first period. A doctor can check whether it is simply your body settling in, or something that needs care.

In this booklet

3	What are PCOD and PCOS?	4	Your monthly cycle
5	The hormones	6	Why PCOD happens
7	Symptoms to watch for	8	Periods: normal or not?
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THE BASICS

What are PCOD and PCOS?



Your ovaries store eggs inside tiny fluid-filled sacs called **follicles**. Each month one follicle usually grows, matures and releases an egg. In PCOD/PCOS, many follicles start growing but **stall**, so the egg is not released on time.

> **Not "cysts" you need to remove**
> The small dots seen on a scan are stalled follicles, not harmful cysts. They do not turn into cancer and usually do not need surgery.

PCOD vs PCOS: is there a difference?

PCOD	Polycystic Ovarian Disease: the popular name. Often described as "milder".
PCOS	Polycystic Ovary Syndrome: the medical name. Often described as "more serious".

Today doctors use one term, **PCOS**, and see it as a **spectrum**. Some women have mild signs, others more. What matters is your symptoms, tests and health risks, not the name.

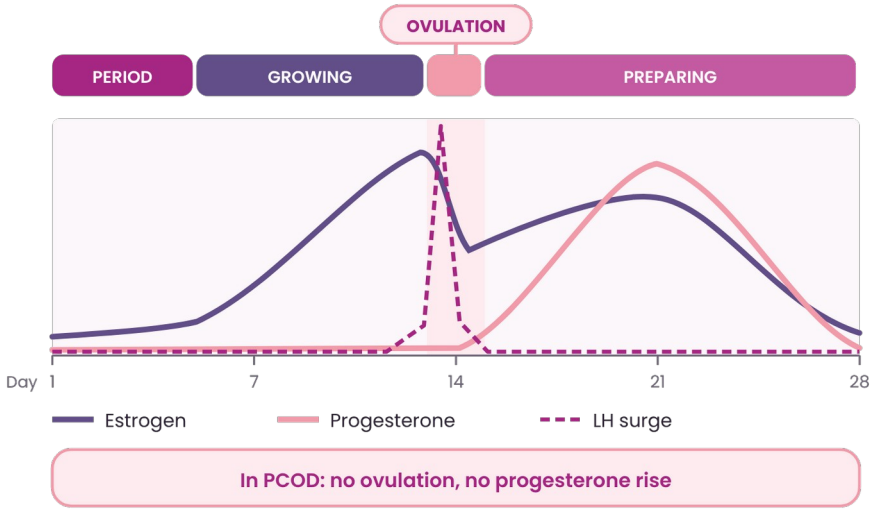
Good to know

- **Not your fault.** Genes, hormones and insulin all play a part.
- **Not contagious** and not caused by one food or one habit.
- **Manageable.** Most symptoms improve with the right care.

YOUR BODY

Your monthly cycle

A typical cycle is a **dance of hormones** between your brain, ovaries and womb. Day 1 is the first day of bleeding.



In PCOD: no ovulation, no progesterone rise

Period	Growing	Ovulation	Preparing
Day 1-5	Day 1-13	About day 14	Day 15-28
The womb lining sheds as bleeding.	Egg sacs grow. Estrogen rises and rebuilds the lining.	A surge of LH releases the egg.	Progesterone prepares the womb. No pregnancy? Period starts.

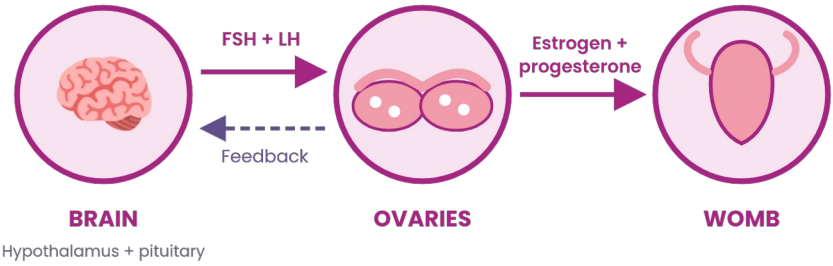
What changes in PCOD?

The egg often **does not release**. Without ovulation there is no progesterone rise, so periods become **late, missed or heavy**, and getting pregnant takes longer.

YOUR BODY

The hormones

Hormones are chemical messengers. The brain sends signals to the ovaries, and the ovaries send signals back. In PCOD this conversation gets out of balance.



FSH

Grows the egg sac each month.

LH

Triggers egg release. Often higher in PCOD.

Estrogen

Builds the womb lining. Supports skin and bones.

Progesterone

Prepares the womb after ovulation. Low in PCOD.

Androgens

Hormones every woman makes in small amounts. Higher in PCOD: acne, hair.

Insulin

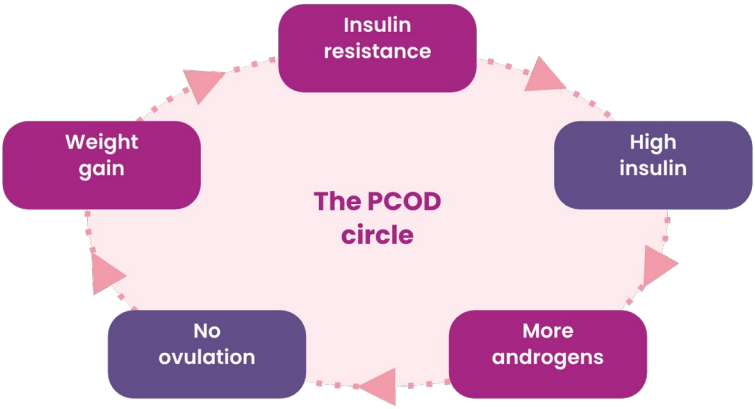
Moves sugar into cells. Often high in PCOD.

Thyroid and prolactin problems can look like PCOD. That is why we always test them too.

THE CAUSE

Why does PCOD happen?

There is no single cause. Several things add up and keep each other going, like a circle.



What adds up

- **Family history:** mother, sister or aunt with PCOD or diabetes.
- **Insulin resistance:** cells respond poorly to insulin.
- **Extra androgens:** ovaries and adrenal glands make more.
- **Lifestyle:** low activity, poor sleep, stress and extra weight can make it worse.

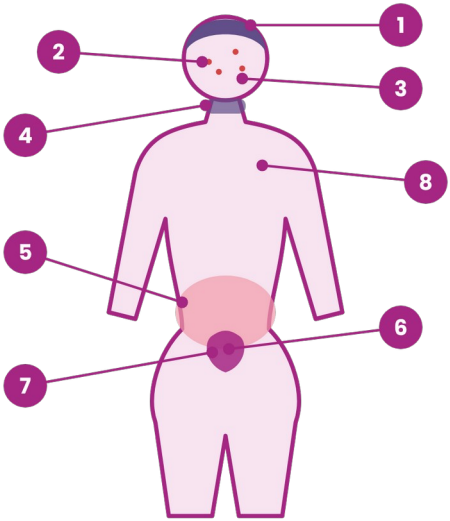
The good news

You can break the circle at **any point**. If you carry extra weight, losing even **5 to 10%** of your body weight can bring back regular ovulation and periods.

SYMPTOMS

Symptoms to watch for

You may have only a few of these. Every woman is different. Numbers match the picture.



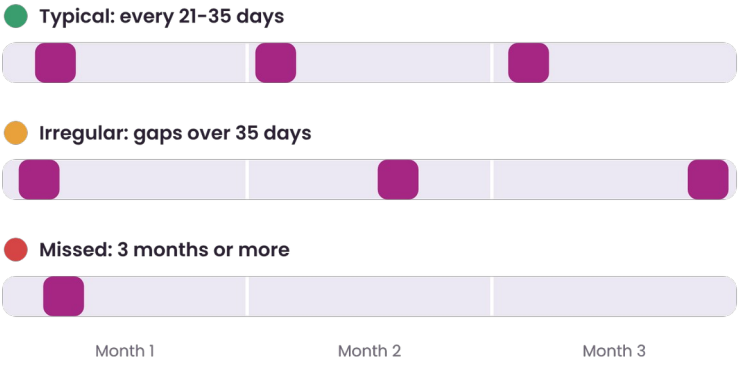
1	Hair thinning at the parting	2	Acne and oily skin
3	Extra dark hair on chin, upper lip	4	Dark, velvety patches on neck, underarms
5	Weight gain, especially around the belly	6	Irregular, missed or heavy periods
7	Difficulty getting pregnant	8	Low mood, anxiety, tiredness, poor sleep

> See a doctor if you notice **two or more** of these, or if one is bothering you.

SYMPTOMS

Periods: normal or not?

Periods are a monthly report card of your hormones. Here is how to read it.



Typical	A period every **21 to 35 days**. Bleeding lasts **2 to 7 days**. A pad changes every 3 to 4 hours or less often.
Worth checking	Cycles **longer than 35 days**, fewer than 8 periods a year, skipping months, spotting between periods.
See a doctor soon	**No period for 3 months** (and not pregnant). Soaking a pad every hour for 2 hours. Large clots. Bleeding over 7 days. Severe pain.

Why skipped periods matter

If periods are missed for many months, the womb lining can keep thickening. Over the years this can cause heavy bleeding and, rarely, serious lining changes. A doctor can give a simple treatment to protect it.

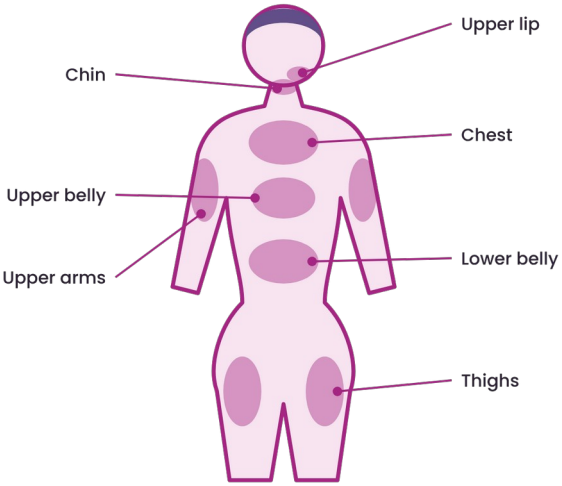
For teens

See a doctor if there is no period by age 15, or more than 3 years after breast growth began.

SYMPTOMS

Hair, acne and hair fall

Androgens make some hair follicles grow thick, dark hair and make oil glands more active. Others, on the scalp, shrink.



Doctors score these areas and the back

Extra hair

Hirsutism

Coarse, dark hair on the upper lip, chin, chest, belly, back or thighs. Different from light, fine facial fuzz that most women have.

Acne

Painful pimples on the jawline, chin and lower face. Often flare before periods and may leave marks.

Hair fall

The centre parting gets wider and hair feels thinner on top. The hairline at the front usually stays.

What helps

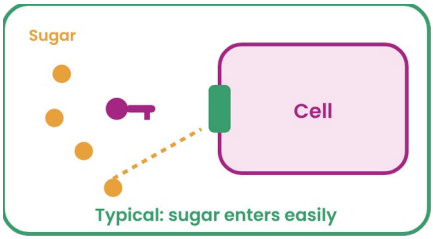
- Treatment takes **3 to 6 months** to show. Hair grows slowly.
- Medicines reduce **new** hair and acne. Laser or electrolysis removes existing hair.
- Shaving or threading does **not** make hair thicker.
- Check **iron, thyroid and vitamin D**. Low levels also cause hair fall.

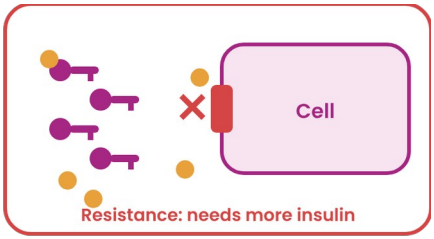
METABOLISM

Insulin and metabolic syndrome

Insulin resistance

Insulin is a **key** that lets sugar into your cells for energy. When the lock gets stiff, the body makes **more keys**. High insulin tells the ovaries to make more androgens.





Signs: dark neck patches, skin tags, sugar cravings, belly fat, sleepy after meals.

Lean women can have it too.

Metabolic syndrome

This means **3 or more** of these 5 are present. It raises the risk of diabetes and heart disease.

Waist	Blood pressure	Fasting sugar	Triglycerides	HDL (good) cholesterol
80 cm or more	130/85 or more	100 mg/dL or more	150 mg/dL or more	Below 50 mg/dL

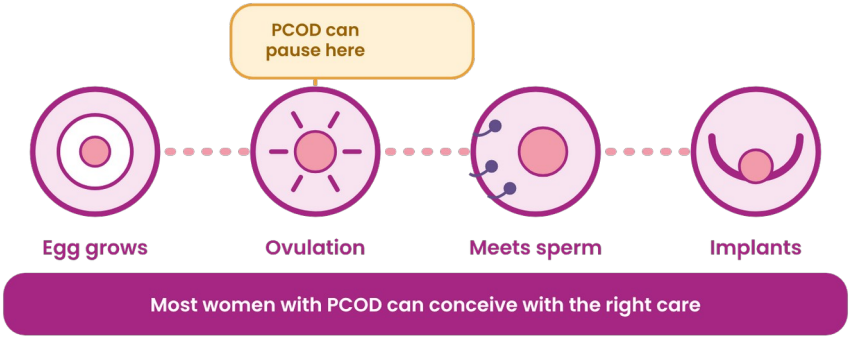
3 of 5 = metabolic syndrome

> A simple blood test and a tape measure are enough to check. Early action can reverse it.

FERTILITY

PCOD and getting pregnant

PCOD is the most common cause of **ovulation problems**. It does **not** mean you cannot have a baby. Most women with PCOD do conceive, sometimes with medical help.



Why it can take longer

- The egg may not mature or release regularly.
- High insulin and androgens disturb the cycle.
- Other causes (tubes, womb, partner's semen) are checked at the same time.

When to ask for help

- Trying for **12 months** (under 35 years).
- Trying for **6 months** (35 years or older).
- **Sooner** if periods are very irregular or absent.

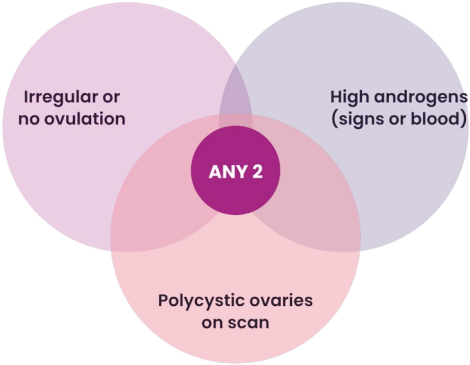
Before and during pregnancy

Start **folic acid** as advised, aim for a healthy weight, stop smoking and alcohol. Women with PCOD have a higher chance of **gestational diabetes and high blood pressure**, so early antenatal care matters.

DIAGNOSIS

How PCOD is diagnosed

There is no single test. Doctors look for **any 2 of these 3** signs, after ruling out other conditions.



Tests you may be advised

Blood	LH, FSH, testosterone, DHEAS, prolactin, TSH, AMH	**Sugar**	HbA1c or fasting sugar, sugar tolerance test, insulin
Heart	Cholesterol and triglycerides, blood pressure	**Vitamins**	Vitamin D, B12, iron, liver tests
Scan	Pelvic ultrasound of ovaries and womb lining	**Body**	Weight, waist, BMI, skin and hair check

> **Teens:** a scan alone is not used to diagnose PCOD in the first 8 years after periods start.

SYMPTOM DECODER

What does it mean?

You notice	It may mean	What to do
Periods over 35 days apart, or skipped	Ovulation is not happening regularly	**Track dates, see a doctor**
Very heavy periods with clots	Thick lining, fibroids or thyroid issue	**Doctor visit soon**
Acne on jaw and chin	High androgens	**Hormone and skin review**
New dark hair on chin or chest	High androgens	**Hormone tests**
Thinning at the parting	Androgens, low iron, thyroid	**Blood tests**
Dark velvety neck patches	Insulin resistance	**Sugar and insulin tests**
Belly weight, sugar cravings	Insulin resistance	**Diet and activity plan**
Tired, snoring, poor sleep	Sleep apnoea, thyroid, anaemia	**Doctor check**
Low mood, constant worry	Common in PCOD	**Talk to someone, counselling**
No pregnancy after 6 to 12 months	Ovulation problem	**Fertility consultation**

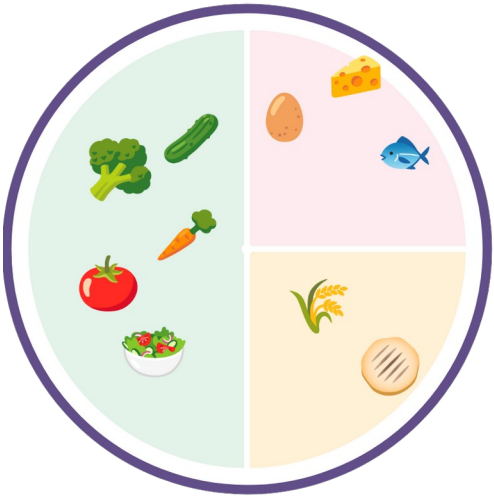
Get help urgently if you have

Severe lower tummy pain, fainting, very heavy bleeding, chest pain or breathlessness, or **sudden** deepening of voice with rapid hair growth.

LIFESTYLE

Eating well with PCOD

There is no one "PCOD diet". The goal is steady sugar levels. Use the plate below at lunch and dinner.



1/2 **Vegetables and salad**

1/4 **Protein**
dal, paneer, egg, curd

1/4 **Whole grains**
millet roti

Healthy fats: a spoon of oil, nuts

2-3 L

Eat more

- Millets: jowar, bajra, ragi
- Dals, sprouts, chana, rajma
- Vegetables and salads
- Curd, paneer, eggs, fish
- Nuts, seeds, guava, apple

Eat less

- Maida, biscuits, white bread
- Sugar, sweets, cold drinks
- Packaged and deep-fried snacks
- Large portions of white rice
- Late-night heavy meals

Small habits that help

- Protein with every meal, especially breakfast.
- Walk 10 minutes after meals to lower sugar.
- Drink 2 to 3 litres of water.
- Avoid crash diets. They backfire.

LIFESTYLE

Move, sleep, calm

Movement makes your cells listen to insulin again. Good sleep and calm balance hunger and stress hormones.

Mon	Tue	Wed	Thu	Fri	Sat	Sun
🚶‍♀️	🏋️‍♀️	🧘‍♀️	🚶‍♀️	🏋️‍♀️	💃	😴
Walk	Strength	Yoga	Walk	Strength	Dance	Rest

🚶‍♀️ 10-minute walk after meals, every day

Move

150 minutes a week of brisk walking, cycling, swimming or dance, plus **2 days of strength**. Even 10 minutes counts.

Sleep

7 to 9 hours, same time each night. Screens off 1 hour before bed. Dark, cool room.

Calm

10 minutes of **yoga, pranayama or meditation** daily. Time for a hobby or friends.

Start small. Pick one habit this week. Small steps you can keep beat big plans you drop.

This week I will:

1.

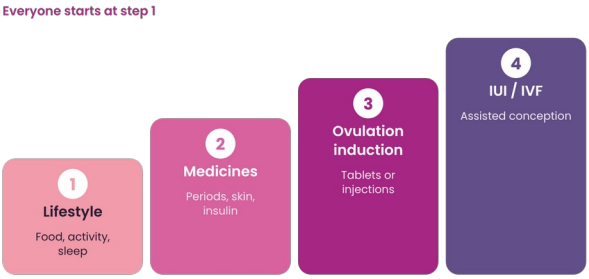
2.

3.

TREATMENT

Treatment options

Treatment depends on **what you want**: regular periods, clearer skin, less hair, or a baby. Your doctor will build your plan.



Your goal	Options your doctor may use
Regular periods, protect the womb	Hormonal pills or a progesterone course
Insulin resistance, weight	Nutrition plan, activity, metformin and other medicines as advised
Acne, extra hair, hair fall	Anti-androgen medicines, skin treatments, laser

If pregnancy is the goal

Ovulation tablets	A short tablet course early in the cycle helps an egg grow and release. Often the first step.
Injections	Used when tablets are not enough. Scans watch the egg growth.
IUI	Prepared sperm are placed in the womb around the time of ovulation.
IVF / ICSI	Eggs are collected, fertilised in the lab, and the embryo is placed in the womb. Used if earlier steps fail or other factors exist.

> **Important:** medicines and supplements (including inositol, vitamin D) are only taken on your doctor's advice. No medicine replaces healthy habits.

WELLBEING

Mind and long-term health

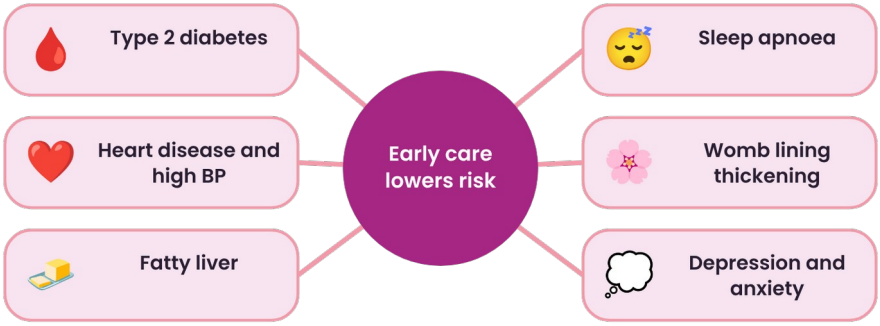
Your feelings are real

Anxiety, low mood and worry about body image are **2 to 3 times more common** in PCOD. It is not weakness. Hormones and insulin play a role. Talking helps. Ask us about counselling.

When to ask for help

Sadness or worry lasting more than 2 weeks, panic attacks, losing interest in daily life, or trouble with sleep or appetite. If you ever think of harming yourself, tell someone and get help right away. Free helpline: **Tele-MANAS 14416** (24x7).

Possible long-term risks



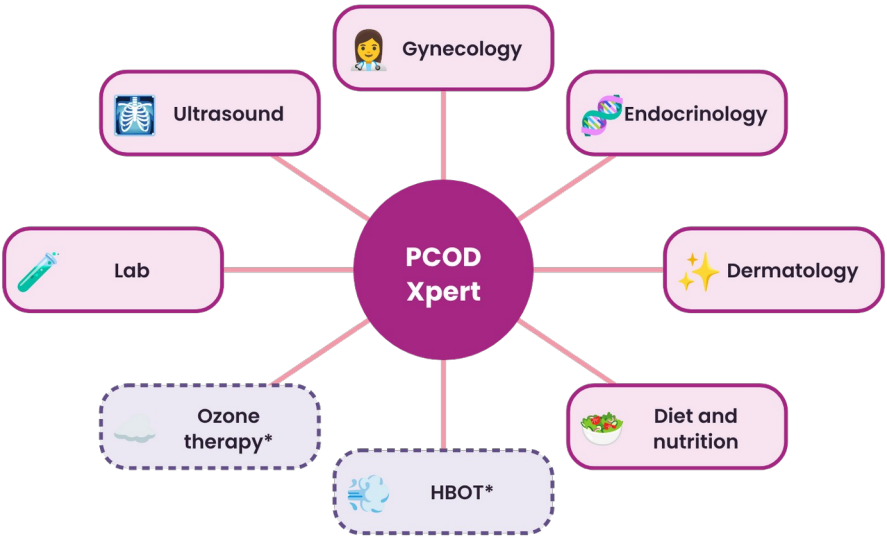
Protect yourself

- Check **blood pressure, sugar and cholesterol every year**.
- Aim for **a period at least every 3 months**.
- Keep active, eat well, sleep well.
- Early care greatly lowers these risks.

OUR TEAM

Everything under one roof

PCOD touches many parts of the body, so one doctor is rarely enough. At PCOD Xpert, a **multidisciplinary team** works together on your plan, from consultation to tests, in one campus.



*supportive, as advised by your doctor

Why one roof helps

- **One plan** shared by all your specialists.
- **Tests and scans** on the same campus.
- **Fewer visits**, clear follow-up and one team to call.

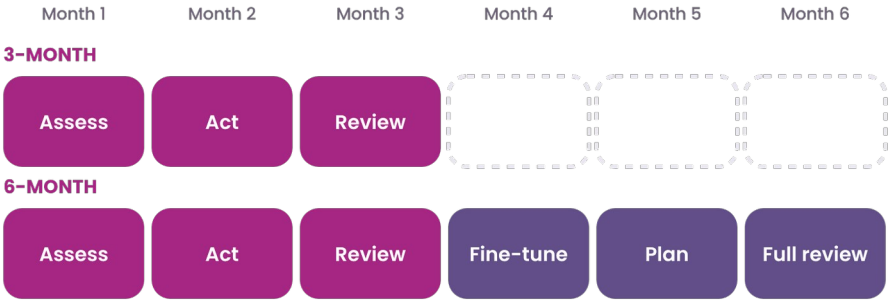
Supportive therapies

HBOT and ozone therapy are offered as **additional supportive options** only when your doctor advises. Research on their role in PCOS is still developing. They do not replace lifestyle, medicines or fertility care.

OUR PROGRAMMES

3 and 6 month programmes

A structured journey with regular reviews, so you are never managing PCOD alone.



3-MONTH PROGRAMME

Month 1	**Assess and plan** Gynecology, endocrine, skin and diet consults. Blood tests and ultrasound. Your personal plan.
Month 2	**Act and adjust** Diet and exercise coaching. Medicines and skin or hair care as needed. Follow-up.
Month 3	**Review** Repeat measurements and tests. Progress check and a plan to keep going.

6-MONTH PROGRAMME

Everything in the 3-month programme, plus **months 4 to 6**: monthly check-ins, fine-tuning of medicines, fertility planning if you wish, skin and hair care cycles, and a full review of your tests and long-term health plan.

Ask our team what is included in each programme for your needs.

Myths and facts

MYTH	FACT
PCOD means I can never have a baby.	Most women with PCOD conceive, sometimes with treatment.
Only overweight women get PCOD.	Lean women can have PCOD and insulin resistance too.
PCOD is caused by junk food.	Genes and hormones come first. Lifestyle helps control it.
Regular periods mean no PCOD.	You can have PCOD with regular periods. Tests tell.
Shaving makes hair grow thicker.	It does not. Hair only looks blunt when it regrows.
I must give up rice and ghee forever.	Portions and balance matter. Crash diets do not work.
PCOD goes away after marriage or a baby.	It may ease for a while, but needs ongoing care.
Pills are the only treatment.	Lifestyle is the base. Medicines are chosen for your goal.

QUESTIONS

Questions and answers

Is PCOD hereditary?

Often it runs in families, but having a relative with PCOD does not mean you will definitely have it.

Will I need medicines forever?

Not always. Many women improve with lifestyle and need medicines only for certain goals or periods of time.

Can PCOD cause cancer?

If periods are very rare for years, the womb lining can overgrow. Treating this early protects you.

Can I exercise during periods?

Yes, gentle activity is fine and may ease cramps.

Can I get pregnant without treatment?

Some women with PCOD ovulate now and then and conceive naturally. If it has not happened in 6 to 12 months, ask for help.

Can stress make PCOD worse?

Yes. Stress hormones affect insulin and ovulation. Sleep, movement and talking to someone all help.

Questions to ask your doctor

- [ ] What type of PCOD do I have?
- [ ] Which tests do I need, and how often?
- [ ] What are my goals for the next 3 months?
- [ ] Which medicines, and for how long?
- [ ] When should I plan a pregnancy?

MY TRACKER

My tracker

Write down the **first day** of every period. Bring this page to your appointment.

Period start	Days	Flow L/M/H	Pain 0–10	Weight	Notes

My medicines and supplements:

Next appointment:

PATIENT STORIES

Hope blooms from within

Real stories from women who took the first step.

PHOTO	" *Add the patient's own words here (with written consent).* **Patient name, age** Years with PCOD / months with PCOD Xpert
PHOTO	" *Add the patient's own words here (with written consent).* **Patient name, age** Years with PCOD / months with PCOD Xpert

PCOD Xpert

Hope Blooms from Within

Don't ignore PCOD.

Connect with us.

Call or WhatsApp us

+91 62625 06050

WhatsApp | Call

Scan to chat on WhatsApp

PCOD & PCOS care under one roof

Gynecology | Endocrinology | Dermatology | Diet & Nutrition | Lab & Ultrasound

Visit us

Sahaj Hospital Campus

15/2 South Tukoganj, Near Manas Bhavan,

Behind Hotel Surya, Indore,

Madhya Pradesh 452005

MEDICAL DISCLAIMER

This booklet is for educational purposes only and is not a substitute for professional medical advice, diagnosis or treatment. Individual results vary. Always consult your doctor before starting or stopping any treatment. Programme details are decided by your treating doctor.